

Title

Bleeding Complications and Clinical Outcomes With Sustained Versus Short-Term Anticoagulant Therapy in Cancer Patients With Pulmonary Thromboembolism

Abstract

Beyond efficacy against recurrent thrombosis, understanding bleeding hazard across patient subgroups is critical for individualizing anticoagulation duration. Among the 178 stratified participants, major bleeding events occurred in 3.1% of the functional-capacity group receiving prolonged therapy versus 8.1% receiving standard therapy, suggesting less bleeding risk with continuation. In the functionally-limited group, prolonged therapy was associated with higher bleeding rates (13.2% vs. 4.7%). Other secondary analyses examined timing of bleeding (most occurred within first 3 months), concomitant cancer therapies affecting bleeding risk, and patient discontinuation due to tolerability. Disease progression and death during follow-up occurred with similar frequency between groups (28% and 27%, respectively). Patient-reported outcomes revealed comparable symptom burden regardless of anticoagulation duration. Subgroup interactions were evident for patients concurrently receiving chemotherapy, among whom extended anticoagulation showed attenuated benefit.